



PRIMODIUM CAPSULES 2MG

Description: Sz 4: Green opaque/grey opaque capsules.

Content:

Loperamide hydrochloride 2mg/capsule

Indication:

For control and symptomatic relief of acute non-specific diarrhoea and of chronic diarrhoea associated with inflammatory bowel disease.

Pharmacology:

Loperamide inhibits intestinal peristalsis by direct effect on the muscles of the intestinal wall.

Oral absorption is incomplete and elimination half-life ranges from 9.1 to 14.4 hours. It is mainly excreted in the feces.

Adverse Effects

Abdominal pain, constipation, dizziness, dry mouth, tiredness, nausea and vomiting and hypersensitivity reactions may occur occasionally. Post-marketing Experience - Cardiac Disorders: QT/QTc interval prolongation, Torsades de Pointes, other ventricular arrhythmias, cardiac arrest, syncope, and death (see Warning and Precautions)

Warnings and Precautions

Caution in patients with dehydration, diarrhoea caused by infectious organisms, and hepatic function impairment. It should be discontinued if abdominal distention occurs or if other untoward symptoms develop in patients with ulcerative colitis. In acute diarrhoea, treatment should be discontinued after 48 hours if improvement does not occur. In chronic diarrhoea, if no improvement has occurred after 10 days of treatment with the maximum dose, loperamide is unlikely to be effective.

Appropriate fluid and electrolyte therapy should be given to protect against dehydration in all cases of diarrhoea. Oral rehydration therapy which is the use of appropriate fluids including oral rehydration salts remains the most effective treatment for dehydration due to diarrhoea. The intake of as much of these fluids as possible is therefore imperative. Drug-induced inhibition of peristalsis may result in fluid retention in the intestine, which may aggravate and mask dehydration and depletion of electrolytes. If severe dehydration or electrolyte imbalance is present. Loperamide should be withheld until appropriate corrective therapy has been initiated.

The use of higher than the recommended doses for control of the diarrhea may cause abnormal heart rhythms and serious cardiac events leading to death. However, in adult patients receiving the recommended dosage of loperamide, cases of syncope and ventricular tachycardia have been reported. Some of these patients were taking other drugs or had other risk factors that may have increased their risk of cardiac adverse reactions. Abuse and misuse of loperamide, as an opioid substitute, have been described in individuals with opioid addiction (see Overdose).

Not recommended for children under 6 years of age. Its use has been associated with fatal episodes of paralytic ileus in infants and young children.

Drug Interactions:

Quinidine or ritonavir (both P-glycoprotein inhibitors) - concomitant administration results in a 2 to 3-fold increase in loperamide plasma levels.

Saquinavir - decreased saquinavir plasma concentrations, and increased loperamide plasma concentrations. Co-administration of loperamide and saquinavir significantly decreases saquinavir C_{max} and AUC, possibly due to impaired absorption of saquinavir caused by the effect of loperamide to the gastrointestinal tract. Due to the possibility of reduced antiviral efficacy of saquinavir, their concomitant use should be avoided, especially for prolonged therapy. There is also an increase in the exposure of loperamide, although it is unlikely to be clinically significant. Itraconazole (inhibitor of CYP3A4 and P-glycoprotein) - concomitant administration results in a 3 to 4-fold increase in loperamide plasma concentrations. Gemfibrozil - increases loperamide by approximately 2-fold. Combination of itraconazole and gemfibrozil - results in a 4-fold increase in peak plasma levels of loperamide and a 13-fold increase in total plasma exposure. Ketoconazole (an inhibitor of CYP3A4 and P-glycoprotein) - concomitant administration results in a 5-fold increase in loperamide plasma concentrations. Oral desmopressin - concomitant treatment results in a 3-fold increase of desmopressin plasma concentrations, presumably due to slower gastrointestinal motility. St John's Wort - delirium with symptoms of confusion, agitation, and disorientation. Valerian - delirium with symptoms of confusion, agitation, and disorientation. Co-trimoxazole - increases the bioavailability of loperamide apparently by inhibiting its first-pass metabolism.

It is expected that drugs with similar pharmacological properties may potentiate loperamide's effect and that drugs that accelerate gastrointestinal transit may decrease its effect.

Contraindications:

Contraindicated in patients with known hypersensitivity to the drug and in those in whom constipation must be avoided.

Use in pregnancy and lactation:

Safety for use in nursing mothers has not been established.

Effects on ability to drive and use machines:

Tiredness, dizziness, or drowsiness may occur in the setting of diarrheal syndromes treated with loperamide HCl. Therefore, it is advisable to use caution when driving a car or operating machinery.

Dosage and administration:

Adults: Oral, 4mg, initially, followed by 2mg after each loose stool, not exceeding 16mg daily. Chronic diarrhoea: initial: oral, 4mg followed by 2mg after each loose stool until diarrhoea is controlled. Maintenance: oral, 4 - 8 mg a day in single or divided daily doses as needed. Children: as directed by physician.

Treatment of over dosage:

Symptoms Depression of the CNS may be seen. In individuals who have intentionally ingested overdoses (reported in doses from 40 mg up to 792 mg per day) of loperamide HCl, prolongation of the QT/QTc interval, Torsades de Pointes, other ventricular arrhythmias and cardiac arrest, have been observed (see Warnings and Precautions). Fatal cases have also been reported. Abuse, misuse and/or overdose with excessively large doses of loperamide, may unmask Brugada syndrome. **Treatment:** Treatment consists of gastric lavage. If appropriate, support of respiration, and use of narcotic antagonists (naloxone), if necessary.

Storage conditions : Store in a cool, dry place below 30°C. Protect from light. Keep out of reach of children.

Presentation/Packing : Capsule 2mg x 100 x 10's

Shelf life : 3 years

Product registration : PRIME PHARMACEUTICAL SDN. BHD.

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